



Barcode No 87139832
Patient Name Mrs. RAJNI
Age/Sex 30 YRS/Female
Referred By DR. POOJA BHAGAT

Lab No 16952408310007
Reg Date 31/Aug/2024 05:39PM
Sample Coll. Date 31/Aug/2024 04:35 PM
Sample Rec. Date 01/Sep/2024 08:42 AM
Report Date 01/Sep/2024 03:51PM

Test Name With Methodology

Quadruple Test

IMMUNOLOGY SPE

Result

Unit

Biological Ref. Interval

AFP.

48.95

ng/ml

Chemiluminescent immunoassay (CLIA)

uE3

2.358

ng/mL

Chemiluminescent immunoassay (CLIA)

Beta Hcg Titer

8965

mIU/mL

Chemiluminescent immunoassay (CLIA)

Inhibin A.

265.6

pg/ml

Chemiluminescent immunoassay

Comments:

Statistical evaluation of Quadruple marker has been done using **Benetech PRA** software.

The test is valid between 15 Weeks to 21 weeks & 6 days of gestation. (optimally performed between 16 and 18 weeks). Quadruple marker test in Second trimester, identifies pregnancies that have a high risk of babies having the following chromosomal abnormalities:

1. NTD (Neural tube defect)
2. Trisomy 21 (Downs syndrome)
3. Trisomy 18 (Edwards syndrome)

Quadruple screening test includes testing for alpha-fetoprotein (AFP), human chorionic gonadotrophin (hCG), unconjugated estriol (uE3), and inhibin A. High AFP levels may indicate that the baby has an open neural tube defect. Lower than normal AFP levels could indicate that a woman is at higher risk for having a baby with Down syndrome. Levels of hCG and Inhibin-A are higher than normal when a woman has an increased risk of having a baby with Down syndrome. Lower than normal levels of Estriol may also indicate that a woman is at high risk for having a baby with Down syndrome.

This Screening test is NOT a diagnostic test (it cannot tell whether the baby has any of the above conditions); it only identifies the risk of the baby being affected. A confirmatory test may be needed when the screening test is positive (high risk). About 5 percent

Prashant K.

Dr. Prashant Goyal
(Chief Pathologist)

A Unit Of Hexamed Healthcare Services LLP

Corporate Office: Flat No 203, 2nd Floor, Sagacity Heights, Plot No 27, Kavuri Hills Rd, CBI Colony, Madhapur, Telangana 500033
Jammu Address: #25-270/6, Main Bypass Road, NH-1A, Channi Rama, Jammu, J & K 180015

✉ feedback@hexamedhealthcare.com @ enquiry@hexamedhealthcare.com 📞 support@hexamedhealthcare.com

☎ +91 95419 01145

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Client	AP081899 Hexamed Healthcare Services LLP		
Code/Name			
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of women have a false-positive result, meaning that the test result is positive but the baby does not actually have Down syndrome.

The interpretive unit is MoM (Multiples of Median) which takes into account variables such as gestational age (ultrasound), maternal weight, race, insulin dependent Diabetes, multiple gestation, IVF (Date of Birth of Donor, if applicable), smoking & previous history of Down syndrome. Accurate availability of this data for Risk Calculation is critical.

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QUADRUPLE MARKER

Patient Name: Mrs. RAJNI
Race: INDIAN
Physician:

Code: 87139832
DOB: 04/04/94
Reported: 01/09/24

CLINICAL INFORMATION

Estimation Method: by U/S (20 wks 6 days on 28/08/24)
Age at Term: 30.8 years
EDD: 09/01/25
Gestation: Singleton
Maternal History: IDDM(U), SMOKER(U), IVF(U)
Gestational Age: 21 weeks 2 days
Referring Lab #: 87139832
Specimen Code: 87139832
Specimen Date: 31/08/24
Received Date: 01/09/24
Weight: 67.0 kg
Screening Status: Initial sample
Para / Gravida: 0 / 0

REMARKS

Down Syndrome

The risk of Down syndrome is LESS than the screening cut-off. No follow-up is indicated regarding this result.

NTD

The maternal serum AFP result is NOT elevated for a pregnancy of this gestational age. The risk of an open neural tube defect is less than the screening cut-off.

Trisomy 18

These serum marker levels are not consistent with the pattern seen in Trisomy 18 pregnancies. Maternal serum screening will detect approximately 60% of Trisomy 18 pregnancies.

BIOCHEMISTRY

MARKER	RESULT	MoM
AFP	48.9 ng/mL	0.71
uE3	2.35 ng/mL	0.95
hCG	8965.0 mIU	0.39
Inhibin A	265.6 pg/ml	1.18

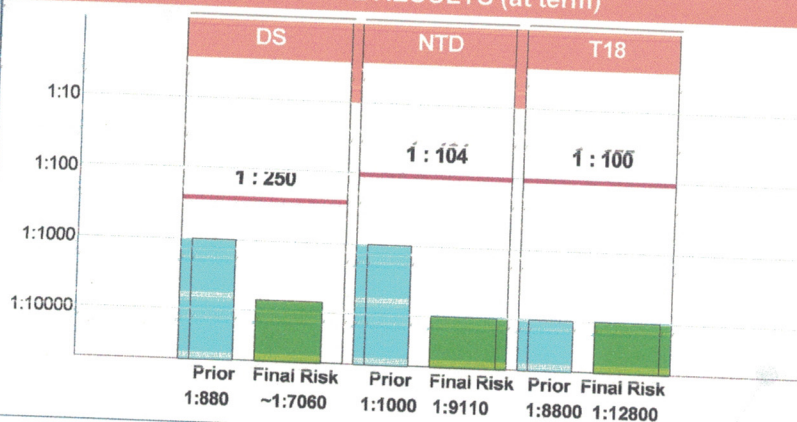
INTERPRETATION

Down Syndrome: Screen Negative

NTD: Screen Negative

Trisomy 18: Screen Negative

CLINICAL RESULTS (at term)



The calculated risk by Benefech depends on the accuracy of the information Provided by the referring physician. Please note that calculation are statistical approaches and have no diagnostic value.

Report Validated by DR
PRASHANT GOYAL

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